SYNTHETIC TEST RECORD - MOCK DATA CREATED FOR CLAIMWISE AGENT VALIDATION ONLY

SUNRISE MULTI-SPECIALITY HOSPITAL & RESEARCH CENTRE

NABH Accredited (Ref No: NABH-H-2022-0941) | Sector 14, Rohini, New Delhi 110085 | Ph: 011-27869000 | TPA Desk: tpa@sunrisehospital-mock.org

DEPARTMENT OF GENERAL & LAPAROSCOPIC SURGERY - DISCHARGE SUMMARY

Patient Name:	Sita Devi	**Age / Gender:**	51 Yrs / Male
IPD Registration No:	IPD-2026-8812	**UHID No:**	UHID-ND-49102
Admission Date & Time:	2026-10-02 09:30 AM	**Discharge Date & Time:**	2026-10-05 01:15 PM
Treating Consultant:	Dr. Arvind Swaminathan, MS, MCh	**Bed / Ward No:**	Bed 312 (Twin Sharing - Ward B)

FINAL CLINICAL DIAGNOSIS

Acute Appendicitis with Localised Peritonitis (ICD-10: K35.8)

CHIEF COMPLAINTS AT ADMISSION

Patient presented with severe right iliac fossa pain for 2 days associated with nausea, low-grade pyrexia, and vomiting. McBurney's point tenderness positive with guarding.

INVESTIGATION SUMMARY

CBC: TLC 14,200/uL, Polymorphs 84%, Hb 13.8 g/dL. USG Whole Abdomen: Non-compressible aperistaltic blind-ended tubular structure in RIF measuring 8.8 mm in transverse diameter with surrounding probe tenderness and minimal serous fluid, confirming Acute Appendicitis.

SURGICAL PROCEDURE PERFORMED

Procedure: 3-Port Laparoscopic Appendectomy under General Anaesthesia on 02/10/2026.
Operating Surgeon: Dr. Arvind Swaminathan | Anaesthetist: Dr. Priya Nair, MD.
Operative Findings: Turgid, inflamed retrocaecal appendix with injected vascularity. Mesoappendix coagulated and divided. Base ligated with Roeder loops. Specimen extracted in endobag and sent for Histopathology Report (HPR).

HOSPITAL COURSE & POST-OPERATIVE RECOVERY

Patient extubated uneventfully. Vitals monitored 2-hourly in post-op recovery. Bowel sounds present on POD-1; sips of water initiated followed by clear liquids. Upgraded to soft diet on POD-2. Surgical wounds healthy, clean, dry, without erythema. Patient ambulating independently without assistance.

CONDITION AT DISCHARGE

Hemodynamically stable, afebrile, tolerating regular oral intake, passing flatus and motions normally. Vitals: BP 120/80 mmHg, Pulse 76 bpm regular, SpO2 99% on room air.

DISCHARGE MEDICATIONS & ADVICE

1. Tab Cefuroxime Axetil 500 mg - 1 tab twice daily after meals for 5 days.
2. Tab Pantoprazole 40 mg - 1 tab once daily before breakfast for 7 days.
3. Tab Paracetamol 650 mg - 1 tab SOS for pain or temperature > 99 F.
4. Clean and dress wound with betadine if required. Keep surgical site dry.

FOLLOW-UP PLAN

Review in Surgery OPD on Day 7 post-op for wound inspection and suture removal. Emergency visit advised if fever, severe abdominal distension, or persistent vomiting occurs.

Dr. Arvind Swaminathan, MS, MCh
Treating Laparoscopic Surgeon (DMC/R/48921)

Dr. K. S. Mehra, MD
Medical Superintendent, Sunrise Hospital